

Applicability:	UW Medicine ¹ and UW Medicine Affiliated Covered Entity ²
Policy Title:	UW Medicine Use of Artificial Intelligence (AI) ³
Policy Number:	COMP.308
Superseded Policies:	N/A
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PURPOSE

The adoption of Artificial Intelligence (AI), including Generative AI (GenAI), presents an opportunity to accelerate UW Medicine's strategic, financial and operational goals. The use of this emerging technology has the potential to improve quality and efficiency across UW Medicine including within the healthcare system and the UW School of Medicine including WWAMI⁴. However, the use of AI poses unique characteristics associated with possible elevated risk. Such risks include, but are not limited to: Privacy

¹ UW Medicine is an integrated clinical, research and learning system with a single mission to improve the health of the public. The clinically integrated parts of UW Medicine consist of the following:

- Airlift Northwest
- Harborview Medical Center
- UW Medical Center
- UW Medicine Primary Care
- UW Physicians
- UW School of Medicine
- Fred Hutchinson Cancer Center is an independent organization that serves as UW Medicine's cancer program (Please note, the UW Medicine Compliance Program and this policy apply to UW faculty. Fred Hutch employees who are not also UW faculty are required to comply with Fred Hutch compliance policies only.)

² The University of Washington (UW) is a hybrid covered entity under the Health Insurance Portability and Accountability Act of 1996 (HIPAA), comprised of healthcare and non-healthcare components. For the purposes of HIPAA, the UW has designated healthcare components and further designates a group of healthcare components to be one affiliated covered entity known as UW Medicine Affiliated Covered Entity (UW Medicine ACE). Healthcare components of the UW Medicine ACE are represented in [101.G1 University of Washington \(UW\) HIPAA Designation – UW Medicine – Affiliated Covered Entity](#).

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³Artificial Intelligence (AI), as referenced within this policy, can refer to an AI System or AI within a tool, application or use case.

⁴ WWAMI is the University of Washington School of Medicine's innovative multi-state medical education program. WWAMI is an acronym for the states served by the UW School of Medicine's M.D. and Graduate Medical Education (GME) programs: Washington, Wyoming, Alaska, Montana, and Idaho.

and security of data; Accuracy and integrity of Algorithm & Model; Bias; and potential impacts on the delivery of medical care and the patient experience. The purpose of this policy is to describe the following: 1) general requirements regarding the use of AI at UW Medicine; 2) when a proposed use of AI requires internal review for assessment and mitigation of potential risks; 3) when a proposed use of AI may be undertaken without review; and 4) additional requirements for Learners.

Out of abundance of clarity, this policy applies to UW School of Medicine.

This policy **does not** apply to Fred Hutchinson Cancer Center.

This policy applies only to research meeting the criteria as defined in Section C.

This policy will evolve over time in response to rapid changes in this innovative technology, the legal and regulatory landscape, and future inclusion of research not currently covered within this policy.

DEFINITIONS – See Attachment A: AI Glossary. All capitalized terms within this policy are defined in Attachment A.

POLICY

A. General Requirements

1. This policy does not replace existing UW Medicine policies and/or processes.
2. The use of any AI must comply with all relevant University of Washington (UW) and UW Medicine policies, including UW Medicine Compliance Patient Information Privacy [policies](#) and [Information Security Risk Management Standard](#).
3. The proposed AI use at UW Medicine must follow the established process for internal review if it meets the criteria set forth in Section B.
4. This policy applies to all actual or proposed AI uses for:
 - a. Clinical Care,
 - b. Education,
 - c. UW Medicine Business Operations (see glossary for definition), and
 - d. Research as defined in Section C.
5. The use of AI has the potential for inaccurate or hallucinated outputs. Users are responsible for reviewing outputs prior to use (e.g., cross-verifying information, complying with Appropriate Use Policies as applicable).

B. AI Uses for Clinical Care, Education, and UW Medicine Business Operations Requiring Internal Review. When a proposed AI use for Clinical Care, Education, or UW Medicine Business Operations includes any of the characteristics described in Section B.1. and is not within the appropriate use scope and parameters outlined on the UW Medicine Approved AI Tool/Application List (see [Vitals](#) for list), then the use is considered to have possible elevated risk and must undergo internal review as described in Section B.2.

1. **AI Characteristics Associated with Possible Elevated Risk:**
 - a. Its output is patient-facing without validation by an individual with the expertise necessary to detect an error (e.g., hallucinations, missing information).

- b. A patient will be directly interacting with it.
 - c. It is trained on, has access to or individuals could input identifiable clinical data (including PHI), including Limited Data Sets.
 - d. It is trained on, has access to or individuals could input UW Medicine De-Identified Health Information that could be accessed by a third party (e.g., vendor or otherwise) or could leave the Secure UW Medicine Environment and has not been certified as de-identified through the Honest Broker process.
 - e. It is trained on, has access to or individuals could input UW Medicine Data (e.g., sensitive data such as legal, financial, IP, PII, confidential/proprietary, trade secret, subject to contractual sharing limitations, academic or Learner data protected under FERPA). (See glossary for UW Medicine Data definition)
 - f. It affects and/or automates Clinical Care (e.g., diagnostics or decision-making) with or without human validation of output.
 - g. It affects and/or automates clinical coding/billing with or without human validation of output, and/or is designed to impact clinical coding/billing.
 - h. It fundamentally changes the work functions of represented (unionized) staff or trainees.
 - i. It requires significant financial investment, exceeding \$100,000 (either one-time cost or annually, including internal labor).
 - j. It affects the selection of candidates for academic appointments, admission, scholarships, awards, or other forms of recognition.
 - k. It presents equity and/or accessibility impacts (e.g., language access, translation accuracy, digital-divide considerations and other impacts as determined when reviewing with the support of the Equity Impact Review Tool).
 - l. It involves a medium or high level of organizational change management to be successful.
2. Internal Review. Proposed AI use that has characteristics associated with possible elevated risk described in Section B.1. will be subject to review by leveraging the UW Medicine IT intake process for Clinical Care, Education, and UW Medicine Business Operations, as described further in Procedures below. If appropriate, more than one review may be required.
- a. AI Use Case Review Council Review. A review will be conducted by the AI Use Case Review Council to ensure relevant risks are identified and approval may be granted with appropriate risk mitigation. The review may include a request for consultations with identified subject matter experts across UW Medicine (e.g., clinical risk, compliance, legal, technical architecture, equity, or contracting representatives).
 - b. AI Education Review Workgroup. An additional review will be conducted for use cases involving Learners or the Learning Environment to consider additional potential risks. The review may include a request for consultations with identified education subject matter experts. For Learners, additional authorization may be required; see Section E.
 - c. AI Operations Subcommittee Review. A review may be triggered by certain characteristics associated with possible elevated risk and/or if appropriate controls cannot be implemented to mitigate risks during the AI Use Case Review Council review.

- C. AI Uses Requiring Internal Review for Research. The UW IRB will conduct a review when research is conducted by a UW principal investigator (PI) that involves:
1. The use of AI, and
 2. Requires UW Institutional Review Board (IRB) review, and
 3. Either UW Medicine Data or targeted enrollment of UW Medicine patients, staff, faculty, or Learners.

The UW Human Subjects Division (HSD) has expanded the definition of human research to include the use of data that is at high risk of re-identification and developed guidance for research involving AI including a required UW Medicine security review. These added components will help the UW IRB ensure relevant AI risks are identified and if appropriate controls are implemented to mitigate the risks. (See UW HSD guidance on [School of Medicine Research using Artificial Intelligence](#) for additional information)

- D. AI Uses Not Requiring Internal Review. Notwithstanding the above, proposed AI uses that do not contain any of the characteristics associated with possible elevated risk described above in this policy are considered 'low risk' and do not require internal review. Use of previously approved tools used consistent with their acceptable use scope and parameters do not require additional internal review. These AI uses must adhere to other existing policies and processes (e.g., [Information Security Risk Management Standard](#) and [Privacy, Confidentiality, & Information Security Agreement \(PCISA\)](#)) and users are still responsible for verifying that outputs are accurate. For Learners, additional authorization may be required; see Section E. Examples of AI uses not requiring internal review include:
1. Any tool listed on the UW Medicine Approved AI Tool/Applications List (see [Vitals](#) for list) and used within the appropriate scope and parameters.
 2. Using a public chatbot (e.g., ChatGPT, Gemini, Claude) to synthesize a published, publicly available scholarly paper concisely for personal review.
 3. Using a public chatbot (e.g., ChatGPT, Gemini, Claude) to develop content, so long as neither the prompt nor any other content contains UW Medicine Data (see glossary for definition) and the generated content is carefully vetted for Accuracy.
 4. Using a public image generator (e.g., DALL-E, NanoBanana) to create an image for a Microsoft PowerPoint presentation.
 5. Using web-based Microsoft CoPilot to create a schedule or agenda for a meeting.
 6. Using an open-source AI data summarization tool (e.g., [LangChain using open-weight models](#)) to analyze publicly available data sets.
 7. Extracting and summarizing information from published articles, and other information available publicly on the internet.
 8. Studying, self-tutoring, practice, or personal reflection, excluding any use involving UW Medicine Data (see glossary for definition).

- E. Additional Authorization for Learners. Use of AI tools by Learners for certain activities requires authorization by the Learner's program, supervisor, or instructor, consistent with [UW's Student Conduct Code](#). Use of AI for the following activities requires authorization, even if using UW Medicine Approved Tools or tools considered "low risk" under this Policy:
1. Coursework. This includes, but is not limited to, class assignments, projects, quizzes, and presentations;
 2. Assessments. This includes, but is not limited to, clinical documentation review, performance ratings, multisource, narrative, oral patient presentation, peer, portfolio-based, practical (lab), project-based, standardized, and simulated;
 3. Exams. This includes, but is not limited to, course or rotation written, computer-based, clinical performance, or oral; and degree milestones and culminating experiences for bachelors, master's and doctoral programs (licensure, preliminary exams, general exams, theses, dissertation, capstones, and meta-projects); and/or
 4. Clinical Learning Environment. Any use of AI in the Clinical Learning Environment as specified under this Policy (see glossary for definition).

PROCEDURES

- A. Internal Review. For AI uses requiring internal review, review is initiated through one of two pathways: IT Project Intake or IRB Intake.
1. IT Project Intake. For Clinical Care, Education, and UW Medicine Business Operations requests, submit the "[New Project Idea](#)" request form.
 2. IRB Intake. To determine if a research study involving AI requires internal review by the UW IRB, complete the [Human Subjects Research Determination Worksheet](#).
 - a. If the worksheet indicates UW IRB review is required, submit a UW IRB Application through [Zipline](#).
- B. Contract Requirements.
1. For Clinical Care, Education, and UW Medicine Business Operations arrangements involving a third party/vendor, a written agreement can only be executed with appropriate UW or UW Medicine delegation of authority after the proposed AI use undergoes internal review and approval. Existing agreements may need to be amended if AI functionality is incorporated after initial execution. For research meeting the criteria as defined in Section C, contracting review and approval processes may occur concurrently with internal review.
 2. The written agreement must be reviewed by the appropriate contracting team (e.g., UW Procurement, UW Medicine Supply Chain, UW School of Medicine Regulatory and Business Affairs, UW Office of Sponsored Programs, etc.) and additional subject matter experts (e.g., Attorney General's Office) to identify risks and ensure contractual safeguards are in place before using the proposed AI product.
 3. If appropriate, the agreement may include an IT Security Terms with AI technology and accessibility requirement exhibits.
 4. Depending on type of UW Medicine Data (e.g., PHI, PII), additional agreements (e.g., Business Associate Agreement (BAA), Data Use Agreement (DUA), and/or Data Processing Agreement (DPA)) and approvals may be required.

C. Security Assessments.

As part of the IT Project Intake or IRB Intake processes referenced in Procedures Section 1A and Section 1B, the UW Medicine Information Security Program must review any AI use under this policy to assess the level of information security risk to the organization. As appropriate, the UW Medicine Information Security Program will conduct an initial and periodic assessment of the security controls in place.

PROCEDURE ADDENDUM(s) REFERENCES/LINKS

- [UW Medicine Compliance Policy COMP.308 Attachment A - AI Policy Glossary](#)
- [COMP.103 Use and Disclosure of Protected Health Information](#)
- [COMP.106 Use and Disclosure of Protected Health Information by Business Associates](#)
- [SS-06 Information Security Risk Management Standard](#)
- [SS-12 Acceptable Use Standard](#)
- [School of Medicine Research using Artificial Intelligence](#)
- [UW Community Standards & Student Conduct: Academic Misconduct](#)
- [Privacy, Confidentiality, & Information Security Agreement \(PCISA\)\)](#)
- [Equity Impact Review Tool](#)
- [Honest Broker Policy](#)

APPROVALS

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8/26/2026
Date